Ensora Health
Practice in harmony.

June 15, 2025

The Honorable Mehmet Oz, MD
Administrator
Centers for Medicare & Medicaid Services
Department of Health and Human Services
P.O. Box 8013
Baltimore, MD 21244-8013.

Re: Request for Information; Health Technology Ecosystem [CMS-0042-NC]

Administrator Oz:

Ensora Health provides innovative, secure software and services to over 200,000 mental and behavioral health therapists. Our AI-enabled solutions span practice management, EHRs, e-prescribing, billing, and more, so therapy providers have everything they need to run their practice efficiently and deliver exceptional care to their clients. Our vision is to cultivate a world with expanded access to quality therapy and mental healthcare. Our mission is to transform mental and behavioral health by streamlining operations, optimizing financial outcomes, and enriching the therapeutic experience for both providers and their clients. Our specialty focus in mental and behavioral health is why we respectfully submit the below comments in response to the Centers for Medicare & Medicaid Services' ("CMS") Request for Information; Health Technology Ecosystem (hereinafter the "RFI") under the Technology Vendors, Data Providers, and Networks subsection.[1]

I. Summary of Ensora Health's Comments

A. The USCDI fails mental and behavioral health providers.

Our position persists that the USCDI fails mental and behavioral health providers on multiple levels. First, USCDI prioritizes general medical and surgical care data elements (*e.g.*, allergies, lab results) at the expense of mental and behavioral health providers. As new versions of the USCDI roll out, Health IT vendors must update their systems to include all newly required data elements, *regardless of whether they're relevant to mental and behavioral health*. Health IT vendors pass those development costs along to customers, either directly or indirectly. Directly through price

[1] Our statements are on behalf of Ensora Health only. Our comments do not necessarily reflect the views of our customers

increases or indirectly by shifting resources to USCDI updates rather than useful improvements and features for the providers. In essence, you're asking a community behavioral health center to subsidize ventilator settings for ICU patients. That's not interoperability, that's inequity.

Second, the USCDI covers a small portion of the data captured and used in mental and behavioral health. Many behavioral health clinicians rely on narrative documentation and formulation models (*e.g.,* SOAP) that don't translate well into rigid fields. As a result, providers who require ONC-certified software for participation in certain programs or grants are often compelled to adopt EHRs designed for primary care workflows rather than therapy sessions. This misalignment results in technology that, at best, fails to meet their clinical needs—and at worst, compromises standards of care and patient safety.

Third, the USCDI adds workflow burden to mental and behavioral health providers. Specifically, they are required to complete templates or click through tabs tied to medical-centric elements to save or submit notes even if those fields aren't used. Non-applicable elements can trigger quality metrics, error alerts, or workflow bottlenecks. Lastly, intake staff, billing teams, and providers waste time navigating non-relevant forms, leading to burnout, delays, and documentation fatigue.

B. Using Artificial and/or Augmented Intelligence (AI) for electronic health information (EHI) exports presents additional risk, especially for Part 2 data and psychotherapy notes.

AI-powered EHI export tools often operate based on generalized rules or pattern recognition and not the nuanced regulatory distinctions that apply to protected categories of health information. Psychotherapy notes, under HIPAA, must be kept separate and excluded from standard disclosures unless expressly authorized by the patient. Also, Part 2 data (substance use disorder treatment records) cannot be disclosed without specific, written consent and are subject to stringent redisclosure limitations. If AI systems automatically export data based on EHR metadata or content without accurately classifying and segregating these protected elements, providers face heightened risk of unauthorized disclosures, violations of HIPAA and 42 CFR Part 2, and potential civil or criminal liability.

C. Harmonizing state reporting would advance interoperability efforts.

While not directly asked in this RFI, we continue to believe that CMS should use its Medicaid authority to compel states to harmonize their reporting requirements. Every state (and sometimes even) asks providers to report different data such as: varying definitions of mental health diagnoses or services; different mandatory fields (e.g., race/ethnicity, treatment type, housing status), distinct file formats, timelines, and transmission methods. This forces Health IT vendors and providers to customize data exports for each jurisdiction, making national interoperability almost impossible.

II. Ecosystem

TD-2. Regarding CMS Data, to stimulate developer interest—a. What additional data would be most valuable if made available through CMS APIs?

Direct access to data could improve agent models in a variety of contexts. Access to denied claims across vendors/provider/payers to see trends where predictions might be made to alert the provider before the initial services is provided that the claim may ineligible for payment and provide steps for the supplier to take to mitigate the risk of denial (add justification, etc.).

III. Digital Identity

D-3. Regarding digital identity implementation—a. what are the challenges and benefits? b. How would requiring digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800-63-3 IAL2/AAL2 CSPs) impact cybersecurity and data exchange? c. What impact would mandatory use of the OpenID Connect identity protocol have?

The utility of digital identity credentials such as CLEAR, Login.gov, or ID.me depends heavily on broad user adoption. Implementing these credentials would incur additional development costs. Any potential benefit would be limited to patients and/or providers who have undergone identity proofing and are currently using interoperability. The cost would not justify the impact at this time or in the next few years. We urge CMS to delay such a requirement on developers until there is a more generally adopted method for digital identity.

Lastly, mandating the use of OpenID Connect would likely decrease the overall usage of interoperability initiatives. Requiring OpenID Connect would impose an additional barrier to adoption by layering complex technical steps on top of an already limited use case for interoperability in many care settings. Regulators should wait to determine if one or more of these technologies will become widely adopted by the general public.

IV. Technical Standards and Certification

TD-7. To what degree has USCDI improved interoperability and exchange and what are its limitations? a. Does it contain the full extent of data elements you need? b. If not, is it because of limitations in the definition of the USCDI format or the way it is utilized? c. If so, would adding more data elements to USCDI add value or create scoping challenges? How could such challenges be addressed? d. Given improvements in language models, would you prefer a non-proprietary but less structured format that might improve data coverage even if it requires more processing by the receiver?

We continue to believe that the USCDI fails mental and behavioral health providers on multiple levels as set forth above in section 1.A. Adding more data elements should only be added after thoughtful consideration of their purpose and audience. Tailoring USCDI for various specialty groups (e.g., Dental, Primary Care, Hospital services, Mental Health, Substance Use and

Recovery, Physical Therapy, Cardiology) would be beneficial. Health IT vendors could then choose the specialties they support. A "base" USCDI could serve as a requirement for all EHR vendors, with additional specialty-specific elements as needed. As such, we continue to propose the following policy changes. First, support modular USCDI compliance, specifically, allowing providers to adopt only what's clinically relevant. Second, allowing a mental and behavioral health–specific certification pathways that focus on narrative, consent, and team-based care. Third, segmented rollouts of medical vs. behavioral data requirements. Fourth, direct engagement with mental and behavioral health clinicians and Health IT developers to shape USCDI development.

With that said, we would not prefer non-proprietary but less structured format. This approach would add new scope, requiring each "document" to be read by an AI model not yet in existence that could accurately understand and display the data in a human-readable format and write to a proprietary database of the receiving system. Regulators should wait before mandating a specific methodology/technology. The industry is already applying different methods and approaches to similar problems but until it is clear how to build and implement these emerging technologies regulators should not make them mandatory. Placing requirements to implement emerging technologies creates a dynamic that will decrease competition in the sector as it will disproportionately burden smaller/newer Health IT vendors.

TD-11. As of January 1, 2024, many health IT developers with products certified through the ONC Health IT Certification Program are required to include the capability to perform an electronic health information export or "EHI export" for a single patient as well as for patient populations (45 CFR 170.315(b)(10)). Such health IT developers are also required to publicly describe the format of the EHI export. Notably, how EHI export was accomplished was left entirely to the health IT developer. Now that this capability has been in production for over a year, CMS and ASTP/ONC seek input on the following: a. Should this capability be revised to specify standardized API requirements for EHI export? b. Are there specific workflow aspects that could be improved? c. Should CMS consider policy changes to support this capability's use?

No. Any change requiring a specific format should have been done prior to moving to production. Adopting a format at this time would add a great amount of scope for limited value to consumers. Work instead should be focused on improving FHIR standards to accept and adopt additional data elements as part of expansion of USCDI to maximize the utility of FHIR with a goal of using FHIR and USCDI to replace B.10.

We underscore the limited utility currently associated with criterion B.10. Consumers even with aid from developers will have limited ability to read the information and providers will not be able to import the data into new vendor systems. This functionality should have the goal of being replaced by FHIR through the increased adoption of additional elements included in USCDI.

CMS should consider no further advancements to this criterion. Any modifications to this criterion would serve only to extract resources from other areas that have a more likely impact in adding value to the consumer.

V. Data Exchange

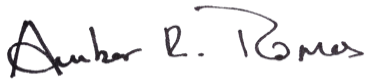
TD-13. What new opportunities and advancements could emerge with APIs providing access to the entirety of a patient's electronic health information (EHI)? a. What are the primary obstacles to this? b. What are the primary tradeoffs between USCDI and full EHI, especially given more flexible data processing capabilities today?

USCDI enables the structured exchange of data that can be accurately interpreted, displayed, and integrated into receiving EHR systems. In contrast, full EHI lacks this structure, limiting its practical use for automated data ingestion and meaningful clinical exchange. Advances may permit in the future the ability for agent models to read non-discrete data, present it to a user (consumer, provider) and add the data to an EHR database but this is not current. Regulators should wait for technology advances before making regulations that if those advances do not occur may not be possible to implement.

VI. Conclusion

We appreciate CMS' consideration of our comments and the opportunity to assist the Agency in its policy mission. Should you have any questions about our comment, please feel free to reach out to us by email at compliance@ensorahealth.com.

Sincerely,



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